

D2 — PCP Telehealth Note (2 wks pre)

Dr. S. Patel, Family Medicine | Primary Care | 02/03/2026

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

TELEHEALTH VISIT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156

Date: 02/03/2026 | Provider: Dr. S. Patel, Family Medicine

Visit Type: Follow-up – Chronic Pain Management

SUBJECTIVE:

Mr. Pemberton presents via telehealth for follow-up of chronic low back pain. Reports pain has been worsening over the past month, particularly with prolonged standing. Rates pain 6/10 at baseline, 8/10 with activity. Current regimen of acetaminophen and ibuprofen PRN provides partial relief. Using cyclobenzaprine at bedtime which helps with nighttime spasms.

Patient also reports using CBD oil (~25 mg daily) which he states "takes the edge off." He purchased this over the counter.

Reports constipation well-managed on current regimen of docusate and senna PRN. Mood stable on sertraline. No other complaints.

ASSESSMENT & PLAN:

1. Chronic low back pain (DDD, lumbar stenosis)

- Continue acetaminophen 500 mg Q6H PRN
- Continue ibuprofen 400 mg Q8H PRN (counseled re: GI/renal risks given CKD)
- Continue cyclobenzaprine 10 mg QHS PRN
- ADD: Lidocaine 5% patch – apply 1 patch daily (12 hours on, 12 hours off) to

lower back

- ADD: Capsaicin 0.075% cream – apply TID PRN to lower back
- Recommend TENS unit for non-pharmacologic pain management
- Noted CBD oil use – documented. Did not formally address at this visit.
- Referral to pain management if no improvement in 4-6 weeks

2. Chronic constipation – well managed. Continue docusate 100 mg BID + senna 8.6 mg QHS PRN.

3. Depression – stable on sertraline 100 mg daily. Continue current dose.

4. All other chronic conditions – stable per recent labs. Continue current medications.

Electronically signed by: Dr. S. Patel, MD

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